

Latex Allergy Management

Policy Number	POL-CLN-046	Owner / Department	Nursing / Perioperative Services
Effective Date	2023-08-21	Version	2.0
Last Revised	2026-01-15	Next Review	2028-01-15
Approved By	Patient Safety & Quality Committee	Applies To	All clinical and support staff, with emphasis on perioperative, procedural, and emergency areas

1. Purpose

To prevent and manage reactions to natural rubber latex (NRL) by identifying at-risk patients and staff and providing a latex-safe environment, consistent with CDC/NIOSH and American Society of Anesthesiologists (ASA) latex-allergy guidance.

2. Scope

Applies throughout the facility and to all patient-care products that may contain natural rubber latex. The facility maintains a low-latex environment and a list of latex-containing and latex-free alternative products.

3. Risk Identification

- Screen every patient for latex allergy at registration and on admission.
- Higher-risk groups: patients with spina bifida or urogenital anomalies, those with multiple prior surgeries, health-care workers, and people with known fruit cross-reactivity (banana, avocado, kiwi, chestnut).
- Distinguish true IgE-mediated latex allergy (Type I) from irritant or allergic contact dermatitis (Type IV).

4. Latex-Safe Management

1. Flag the allergy in the medical record, wristband, and door/room signage.
2. Remove latex-containing items from the patient's environment and substitute latex-free alternatives.
3. Schedule procedures for a latex-allergic patient as the first case of the day to minimize airborne latex-protein exposure.
4. Use latex-free gloves, tourniquets, IV ports, and medication vials/stoppers; stage a latex-free cart.
5. Communicate latex precautions at every handoff and to all team members before the procedure.

IMPORTANT — A latex-allergic patient presenting with an acute Type I reaction — urticaria progressing to wheeze, stridor, or hypotension — is a medical emergency: remove all latex source items immediately, give intramuscular epinephrine 0.3 mg, call a Code Blue (dial 55), and provide oxygen and IV fluids.

5. Staff Protection

- Provide powder-free, low-protein or non-latex gloves to reduce sensitization.
- Refer staff with suspected latex allergy to Employee Health for evaluation and accommodation.
- Report occupational latex reactions through the staff-safety process in POL-RM-013.

6. Severity & Reporting

Report latex exposures and reactions per POL-RM-013.

Severity	Example	Notify / timeframe
SEV-1	Latex-induced anaphylaxis with cardiac arrest, death, or permanent harm; latex exposure of a documented latex-allergic patient causing severe reaction (sentinel event).	Attending, Administrator-on-Call, and Risk Management within 1 hour.
SEV-2	Latex reaction requiring rescue treatment with major temporary harm.	Attending and Risk Management within 4 hours.
SEV-3	Minor localized reaction (e.g. contact dermatitis) with no systemic harm.	Unit manager within 24 hours via event report.
Near Miss	Latex item staged for an allergic patient but caught and removed before use.	Event report within 24 hours.

7. References

- CDC / NIOSH, Preventing Allergic Reactions to Natural Rubber Latex in the Workplace.
- American Society of Anesthesiologists (ASA) guidance on latex allergy.
- Related: POL-CLN-045, POL-RM-013.